

جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إِسْمُ الْمَرِيضِ رُبَاعِيًّا : امانى محمد عيسوى ميهوب

المحافظة : الفيوم الحي / المدينة : الفيوم

رقم البطاقة : 29607172300961 رقم التلفون : 01129196939 تاريخ تسجيل الإستمارة : 27-09-2026

العمر(تاريخ الميلاد) : 17-07-1996 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----)

المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : جراحات أورام تدخل جراحی لاستئصال كامل /تحفظى للثدى +تفريغ قسم العلاج :
الاورام الغدد الليمفاوية و/ او الغدد الحارسة [13155]

----- التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الإسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: _____ إلتوقيع:

الوظيفة: ----- التوقيع:

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- /د ----- /د ----- /د.ا ----- /د.ا

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جھة العلاج : --

ملاحظات : ---

ختم

شعار الجمهورية

مدیر عام

مديرية الشئون الصحية

رئيس

المجلس الطبي المتخصص

Name: امانى محمد عيسى ميهوب
National ID: 29607172300961



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الى Mastectomy or breast-conserving surgery with axillary lymph node
dissection and/or sentinel lymph node biopsy

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



 المملكة العربية السعودية وزارة الصحة 04/12/2025	
MDT FORM	
Name: الاسم: محمد بن محمد	MRN: 9742
Age: 30 Y	ID: 29607172300961
Menopausal Status: PERMENOPUSAL	Phone: 01129190939
NRS SCORE:	Marital Status: MARRIED 2 MALE 1 FEMALE
BMI:	Family History: +VE FB ANUNT
CO Morbidity:	Reason Of MDT:

Clinical Data:

BX: IDC G2-3
ER 0/8 **PR** 0/8 **HER** C/B **KI67** 70%

Sonomammography:Mets Workup:History:

a case of female pt 29 yrs RT LABC TNBC path -->
 IDC G2-3 ER 0/8 ; PR 0/8 ; Her2 -ve & Ki67 70% CT3
 N1

PET CT --> free

Not mets

pt insert Clips

Recommendations:

FOR START IMMUNOTHERAPY Taxol + CARBO X4
 CYCLES THEN AC X4 CYCLES

MDT Outcome & Plan

START IMMUNOTHERAPY Taxol + CARBO X4 CYCLES
 THEN AC X4 CYCLES THRN BCS + SLNB

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					

Name: اماني محمد عيسوي ميهوب			
MR. : 9742			
Detailed Medical Report			
			السيد
تحية طيبة وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمناظرة وجد انه A 29-year-old patient with right-sided, T3, N1, triple-negative breast cancer (IDC G2-3, ER 0/8, PR 0/8, HER2-negative, Ki-67 70%) underwent preoperative staging with PET-CT, which showed no distant metastases. The patient received surgical clips for multidisciplinary team discussion and was planned to start neoadjuvant therapy with a regimen of pembrolizumab plus taxane-carboplatin for four cycles followed by four cycles of doxorubicin-cyclophosphamide (AC). The first cycle of therapy was administered as taxol 155 mg/m² and carboplatin AUC 2 (250 mg) with pembrolizumab, with laboratory work pending. Subsequent visits documented the continuation of this regimen: cycle 1 week 2 taxol 155 mg/m² and carboplatin AUC 2 (250 mg) with pembrolizumab; cycle 2 week 4 taxol 155 mg/m² and carboplatin AUC 2 (250 mg) with pembrolizumab, noting a creatinine of 0.48 mg/dL, hemoglobin 10.3 g/dL, platelets 167 × 10⁹/L, and total lymphocyte count 3.6 × 10⁹/L. Supportive care included silymarin and prophylactic nebulized antibiotics. The patient reported urinary tract infection symptoms during weeks 5 and 6. Cycle 2 week 5 taxol and carboplatin AUC 2 were administered with pembrolizumab. Cycle 6 week 6 taxol 155 mg/m² and carboplatin AUC 2 were given. During week 7, the patient received taxol 155 mg/m² and carboplatin AUC 2 over 2 hours with premedication of soluortef and avil; a grade 2 hypersensitivity reaction to carboplatin was observed. The same regimen was repeated in week 8 with the addition of a prophylactic G-CSF following a complete blood count. The patient received two weeks of taxol and carboplatin outside the clinic for week 12, followed by a transfusion of one unit packed RBCs and a plan to resume AC plus pembrolizumab at the next clinic visit. Cycle 4 of pembrolizumab (200 mg) was administered on 6 July, with the patient weighing 87 kg and measuring 165 cm; an echocardiogram on 6 July showed an ejection fraction of 68 %. Cycle 1 of AC (adriamycin 120 mg/m² and endoxan 1200 mg) was initiated on 9 July. Cycle 2 of AC (adriamycin 120 mg/m² and endoxan 1200 mg) was given on 30 July, with a concurrent cycle 5 of pembrolizumab; laboratory results showed a two-fold increase in AST, for which silymarin was advised. By 20 August, imaging demonstrated a complete radiological response, and the patient was noted to have a BRCA1 mutation. On 5 September, cycle 6 of pembrolizumab was administered, with the next AC cycle scheduled for 10 September. On 10 September, the patient received 12 weeks of taxol/carbo, three cycles of AC, and six cycles of pembrolizumab, and was evaluated for surgical assessment. The patient has completed 12 weeks of taxol/carbo, 3 cycles of AC, and 6 cycles of pembrolizumab to date.			
تحريراً في ,			
التاريخ : 21-09-2026		الوقت :	
الطبيب المعالج :		شريف الشناوي	
يعتمد ,			

4/12/26, 3:18 PM

code 02 - Name_AMDT - Name_EMDT



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية



04/12/2026

MDT FORM

Name:	Age:	MRN:	9742
Menopausal Sta:	PERIMENOPUSAL	ID:	29807172300961
NBS SCORE:		Phone:	01129198939
BMI:		Marital Status:	MARRIED 2 MALE 1 FEMALE
CO Morbidity:		Family History:	+VE FH ALZHEIMER
		Reason Of MDT:	

Clinical Data:

BX:	IDC G2-3
ER:	0/8
PR:	0/8
HER2:	0/8
Ki67:	70%

Sonomammography:

Mets Workup:

History:

A case of female pt 29 yrs RT LABC TNBC path --> IDC G2-3 ER 0/8, PR 0/8, Her2 -ve & Ki67 70% CT3 N1

PET CT --> free

Not mets

pt Insert Clips

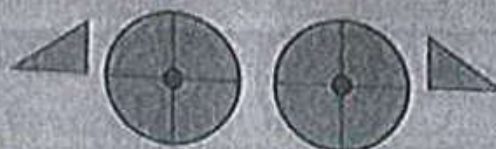
for MDT for start Pernop + Taxol + Carb x 4 then AC x 4

Recommendations:

FOR START IMMUNOTHERAPY Taxol + CARBO X4 CYCLES THEN AC X4 CYCLES

MDT Outcome & Plan

START IMMUNOTHERAPY Taxol + CARBO X4 CYCLES THEN AC X4 CYCLES THEN BCS + SUNB



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
------------------	---------	-----------	-----------	-----------	-------------

04/12/26
04/12/26

Patient Name : الأستاذ / أماني محمد عيسى ميهوب
Patient No. : 29607172300961
Path No. : SR-409/26
Sex : Female
Age : 29 Y
Referring Doc :
Request No. : 13005630
Request Date : 30/03/2026
Verification Date : 04/04/2026
Financial Category : نفقة الدولة - جوسف رومي
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

- One referred block labelled by (545-26). Together with, five referred slides including one H&E stained slide and four immunostained slides for (ER, PR, HER2 & Ki67).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * DCIS: Minimal high grade ductal carcinoma in situ (Solid pattern).
- * Angiolysis: Absent.
- * Perineural invasion: Absent.
- * Necrosis: Focal.

* IPT:

- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Negative membrane reaction in tumor cells (score 0).
- Ki67%: 70%.

Diagnosis:

- Right breast mass, Referred block & slides (Tru-cut biopsy):
- Invasive duct carcinoma, Grade 3.
- Minimal high grade ductal carcinoma in situ (Solid pattern).

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).

Verified by :

Dr.Duaa Mohamed

Printing Date : 09/04/2026 09:09:26

Page 1 of 2

Printing By : marwa mohamed youss



Patient Name : الأستاذ / اماني محمد عيسى ميهوب
Patient No. : 29607172300961
Path No. : SR-409/26
Sex : Female
Age : 29 Y
Referring Doc :

Request No. : 13005630
Request Date : 30/03/2026
Verification Date : 04/04/2026
Financial Category : نفقة الدولة - جوستاف روسي
Department : Out Patient
Registered by : .marwa mohamed youss

Pathology Report

- Her2: Negative (score 0).
- Ki67: 70%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Prof. Dr. Maha Guimei.
Dr. Heba El Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 09/04/2026 09:09:26

Page 2 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



FAYOUM ONCOLOGY CENTER

مركز أورام الفيوم

٩٧٤٤

محافظة الفيوم - مديرية التضامن الاجتماعي
الجمعية العلمية لرعاية مرضى الأورام
المشهرة برقم 356 لسنة 2000

Patient name:	AMANY MOHAMED EYSAWY	Age:	29 years
Date of exam:	2026/3/17	Gender:	female
Ref Physician:	Prof. Dr.		
Examination:	Breast MRI		

Technique:

Axial T2 with fat suppression.

Axial T1 fl-3D dynamic with fat suppression Sequences & (with subtraction).

Clinical Background:

Palpable right breast lump. Pathology reported an IDC,

Comparison: None.

FINDINGS:

- Bilateral prominent fibroglandular tissue noted showing moderate, rather symmetrical background parenchymal enhancement.
- A heterogeneously enhancing, irregular mass lesion, showing angular margins, with associated surrounding area of non-mass enhancement, are seen centered on the upper and lower outer quadrants of the right breast (at an approximate distance of 6.3 cm from the ipsilateral nipple). It presents a low to intermediate signal intensity on both T1 and T2WIs and a high signal intensity on STIR images, with multiple areas of breaking down seen within. It measures approximately 5.2 X 3.8 X 3.7 cm along its maximal AP, CC and transverse dimensions, respectively. Surrounding distortion and edema signal are noted.
- Upon performing the dynamic post contrast series, the aforementioned lesion presented a type II curve showing a slow initial phase of contrast uptake, followed by a plateau delayed phase.
- Normal skin thickness and contour.
- Normal nipple-areola complexes.
- Few, right axillary, globular lymph nodes are seen showing non-uniform cortical thickening and effaced/ muffled hila.
- Left axillary lymph nodes show average cortical thickness and preserved hila.
- Intact chest wall.



Opinion:

Right breast pathology-reported IDC. The current study shows right breast mass lesion showing extensions, morphology and kinetics as above described with associated ipsilateral pathological axillary lymphadenopathy (BIRADS 6).

Dr. Dalia Saleheldin, MD

شفاء وأمان

د. داليا السعيد - خلف مستشفى الصدر - الفيوم - مصر

الخط الساخن: 16382

فون: 0842184174

بريد الإلكتروني: info_foc@focegypt.com



وحدة دلالات الأورام
قسم الباثولوجيا - قصر العيني
ت : ٠١٠٦٨٥٢٩٦٨٤

IMMUNOHISTOPATHOLOGY

Patient Name: أمالي محمد عيسى

lab.No:2026-356

Date:15/3 2026

Age:29Y

Sex: Female

Markers: ER, PR, Her 2 neu and Ki67 .

Procedure:

Sections were prepared from paraffin blocks treated for (ER-clone SP1), (PR clone 1E2), (Her 2 neu-clone 4B5) and (Ki67- clone 30-9) automatically stained in Bench mark ULTRA (VENTANA) using VIEW DAB Detection Kit, Hematoxylin as counter stain

Results:

Examination of sections treated for ER revealed no nuclear staining of tumor cells by chromogen.

Examination of sections treated for PR revealed moderate nuclear staining 10% of tumor cells by chromogen.

Examination of sections treated for Her 2 neu revealed no membranous staining of tumor cells by chromogen

Examination of sections treated for Ki67 revealed positive nuclear staining in 80 % of tumor cells by chromogen.

DIAGNOSIS: RT. Breast mass, TCNB;IPT

- NEGATIVE FOR ER SCORE (0/8)
- POSITIVE FOR PR SCORE (4/8)
- NEGATIVE FOR HER 2 NEU SCORE(0) NULL
- Ki67: 80%

PROF. DR. SAMIA GABAL
PROF. DR. MONA SALAH

(Signature)

Alfa
CURE CENTER

oncology center



أ. د. هشام الفزالي

استاذ علاج الأورام - طب عين شمس

مدير مركز أبحاث طب عين شمس MASRI

رئيس الجمعية المصرية للأورام ECS

الاسم: أ/د/ هشام الفزالي

التاريخ: ٢٠١٤/٢٧/٢٠

R1

Beast Garden

line Triple rue

for Kaytunda 2000

Taxol
carb.

مركز الفزالي
لعلاج الأورام

١٥٨٥٩

القاهرة

علاج سرطان: ١٥٨٥٩ شارع الخديوية بجوار بنزلة هوسا

ت: ٠٢ ٤٦٩٠٠١٦٦ - ٠١٠١١٥٤١٣١ - ٠١٠١١٥٤١٣٠

مصر الجديدة: ١١٦ شارع النهضة - ميدان تريومف (أمام مدرسة ليليه الخديوية)

ت: ٠١٠١١٥٤١٣٥ - ٠١٠١١٥٤١٣٠ - ٠٢ ٤٤١٤٤١١٠

الجيزة

الهندسين: ٥ شارع جامعة الدول العربية آخر تون لنادي الزمالك

ت: ٠١٠١١٥٤١٣١ - ٠١٠١١٥٤١٣٠ - ٠٢ ٤٤١٤٤١١٠

الإسكندرية

شارع قنطرة: ١٢٧ طريق الحرية - أول شارع غزاد - أمام ساحة الزهور - باب شرق

ت: ٠١٠١١٥٤١٣٧ - ٠١٠١١٥٤١٣٦ - ٠٢ ٤٤١٤٤١١٠

١٥٨٥٩

Alfa Cure Oncology Centers www.AlfaCureCenter.Com

Dr. Mohamed H. Kobaisi, MD.
Constiant Pathologist



معمل الأهرام للفحائيل
الطبية والباثولوجية

د/ محمد حامد قبيسي
دكتوراة تحاليل الأنسجة
والأورام والخلايا - القصر العيني

Patient name	: أماني محمد عيسى ميهوب	Lab No.	: 11829
Age	: 29 Year 7 Month 11 Day	Barcode No.	: 2603HP06037
Sex	: Female	Received at	: 03/03/2026 11:07
Referred by	: Dr. Ahmed Ragab	Reporting issued at	: 04/03/2026 16:22

HISTOPATHOLOGY REPORT

CLINICAL DATA:

- * Right Breast Mass.
- * TCNB.

GROSS PATHOLOGY:

Three grayish white tissue cores measured 3.5 cm in total collective length, totally submitted in 1 cassette.

HISTOLOGY:

Sections shows mammary parenchymal tissue cores with infiltrative clusters and cords made up by moderately to markedly anaplastic ductal cells. Stroma is desmoplastic.

DIAGNOSIS:

- Right Breast Mass; Tru-Cut Needle Biopsy:
- * INVASIVE DUCT CARCINOMA GRADE II-III
 - * NO REMARKABLE DCIS RECORDED

Signature
Dr. Mohamed H. Kobaisi

L fe

Life Scan Center

RIGHT BREAST SONOGRAPHY

AMANY MOHAMMED ESAWI

"2-Mar-26"

- Well defined solid lesion within outer aspect of the breast measuring about 4.8x2cm seen wider than taller and parallel to the skin showing few cystic changes within.
- No nipple retraction or areolar thickening.
- Normal thickness/density of subcutaneous layer.
- Fair trabecular pattern with no retro-areolar ductal ectasia.
- No cystic masses.
- No abnormal axillary lymph nodes.

CONCLUSION

➔ Large mass lesion within outer aspect of the breast (BIRADS 4A), for further biopsy and histo-pathological assessment.

Dr./Ahmed Bakkar Hamed

- BI-RADS 1: negative
- BI-RADS 2: benign, 0% probability of malignancy
- BI-RADS 3: probably benign, <2% probability of malignancy
- BI-RADS 4: suspicious for malignancy
 - BI-RADS 4A: low suspicion for malignancy (2-9%)
 - BI-RADS 4B: moderate suspicion for malignancy (10-49%)
 - BI-RADS 4C: high suspicion for malignancy (50-94%)
- BI-RADS 5: highly suggestive of malignancy
 - >95% probability of malignancy



Patient's Name: - Amani Mohamed Eisawi Mayhoub

Date: -10/3/2026

Whole body FDG PET-CT examination

Clinical background:

- 29-year-old female.
- Presented with right breast pathologically proven invasive ductal carcinoma.
- No available previous studies for comparison.
- Weight = 86 KG, BL glucose level = 99mg % , Height=165cm

Reason of study:

- Initial Assessment.

Radiopharmaceutical:

- F-18-FDG 5.11 Mci injected.

Technique:

- Approximately one hour after IV injection of the radiopharmaceutical, whole body PET-CT examination from the skull to mid-thigh was performed with arms extended above head.
- Low dose non-contrast CT was acquired for the purpose of attenuation correction and anatomical localization.
- Data was processed and displayed according to department protocol.
- Calculation of SUV-max was done at region of interest, using average hepatic uptake and blood pool uptake as a reference.
- Average hepatic uptake ~ 1.09
- Average blood pool uptake ~ 0.93
- No IV contrast was given.



Patient's Name: - Amani Mohamed Eisawi Mayhoub

Date: -10/3/2026

Findings:

↓ Reviewing PET, CT and fused PET-CT images:

I. Breast and axilla:

- Right breast:
 - Increased FDG uptake by upper outer quadrant lesion with speculated outline with SUV max 24.3 and measures 4.8cm x 3.9cm.
 - Smaller similar lesion is noted in the lower outer quadrant with SUV max 7.8 and measures 1.2cm.
 - No skin thickening or chest wall infiltration
- Increased FDG uptake by right level I axillary lymph nodes with SUV max 6.7 and measures 2.5cm.
- Left breast: no FDG avid lesions.

II. Brain:

- Diffuse symmetrical trace uptake is seen along the examined brain with no metabolically active (FDG avid) lesions could be detected.
- Normal size and configuration of the ventricular system with no midline shift.
- No cerebral parenchymal areas of abnormal attenuation value.
- No areas of fresh blood density seen intra or extra axial.
- Normal posterior fossa structures.



Patient's Name: - Amani Mohamed Eisawi Mayhoub

Date: -10/3/2026

III. NECK:

- No metabolically active (FDG avid) cervical lymph nodes.
- No metabolically active (FDG avid) neck masses.
- Physiological symmetrical tracer uptake at salivary glands with no metabolically active (FDG avid) lesions.
- Normal appearance of the aerodigestive air column with no parapharyngeal masses.
- Normal appearance of the vocal cords, both pyriform sinuses and ary-epiglottic folds. No glottic, supra- or infraglottic masses. Intact laryngeal skeleton.
- Normal CT appearance of the thyroid gland.

IV. CHEST:

- No metabolically active (FDG avid) pulmonary nodules or masses.
- No metabolically active (FDG avid) mediastinal lymph nodes.
- No metabolically active (FDG avid) pleural effusion or masses.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

V. ABDOMEN AND PELVIS:

- The liver is average in size with homogenous CT texture. No definite metabolically active (FDG avid) focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Average size spleen showing no metabolically active (FDG avid) focal lesions with average physiological FDG uptake.

Patient's Name: - Amani Mohamed Eisawi Mayhoub

Date: -10/3/2026

- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- No metabolically active (FDG avid) abdominal or pelvic lymphadenopathy.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No metabolically active (FDG avid) abdominal or pelvic collections.



Patient's Name: - Amani Mohamed Eisawi Mayhoub

Date: -10/3/2026

VI. Scanned axial and proximal appendicular skeleton:

- No metabolically active (FDG avid) osseous lesions could be detected.

Conclusion:

PET-CT study for a case of right breast cancer on initial assessment showing:

- Metabolically active (FDG avid) right breast neoplastic process associated with metabolically active (FDG avid) metastatic right axillary lymph nodes as described.
- No definite metabolically active (FDG avid) distant metastatic lesions.

Best Regards;

Dr. Ayman Abo El-Regaf; M.D
Radiology consultant- NCI - Cairo University

Prof Dr. Khaled Elshantafy
Professor of radiology Cairo University

د. رانيا
رئيسة
قسم
الجراحةمبادرة صحة المرأة المصرية
لدعم صحة المرأة المصريةمبادرة
صحة
المرأة
المصرية

09/23/2026

MDT FORM

Name:	الاسم: محمد عيسى مظهر	MRN:	9742
Age:	30 Y	ID:	29607172300961
Menopausal Sta	PRE	Phone:	01129196939
NRS SCORE:		Marital Status:	
BMI:		Family History:	+VE FH AMUNT
CO Morbidity:		Reason Of MDT:	

Clinical Data:

R

BX:	IDC G2/G3						
ER:	0/8	PR:	0/8	HER2:	0	KI67:	70%

Sonomammography:

Mets Workup:

History:

a case of female pt 29 yrs RT LABC TNBC path --> IDC G2-3 ER 0/8 , PR 0/8 ,
Her2 -ve & KI67 70% CT3 N1
received NACT >> 12w taxol/carbo + 6c keytruda + 4c AC last 10/9/2026
sm on 19/7/2026 complete radiological response ((((((brca 1 mutant))))))
NEW CESM:
RT MIDDLE 1/3 UOQ AREA OF ARCHITECTURE DISTORTION WITH BREAST
CLIP INSERTED DENOTING COUPLE RADIOLOGICAL REPONSE
LT BREAST : NORMAL
pt is candidate for rt NSM + SLNB and LT prophylactic NSM + reconstruction
omnia essay

TNM

Recommendations:

MDT Outcome & Plan

In view of having BRCA 1 mutation...and after pt counselling, consider rt NSM
+ SLNB and LT prophylactic NSM + reconstruction.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. شيماء لاشين	د. شيماء عيسى				



Patient Name: Amany Mohamed Ewisy

Date: 17/09/2026

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: 30 years old female patient with right pathologically proven IDC grade III (-ve ER, -ve PR, -ve HER2 and Ki67= 70 %) CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild background enhancement

Heterogenous dense

RIGHT BREAST:

- **Low energy (Non contrast) images:** The right breast shows middle 1/3 UOQ area of architectural distortion with clip inserted within and about 7.8 cm from the nipple.
- **Recombined (Post contrast) images:** corresponding no pathological uptake with clip inserted within denoting complete radiological response.
 - Normal skin / nipple / areolar complex.
- **Complementary US:**
 - The right breast shows middle 1/3 UOQ area of parenchymal distortion with clip inserted within.
 - Preserved axillary lymph nodes

LEFT BREAST:

- **Low energy (Non contrast) images** No masses, asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images** show no pathological uptake.
 - Normal skin / nipple / areolar complex.
- **Complementary ultrasound:**
 - Preserved axillary lymph nodes

IMPRESSION:

- **RIGHT BREAST:** (BIRADS 6= pathological proven) Right middle 1/3 UOQ area of architectural distortion with clip inserted denoting complete radiological response as described.
- **LEFT BREAST:** (BIRADS 2= benign) Normal study.

Best regards,
Dr. Hedaya Nour, MD